

--SUMMARY--

Decision No. 293/17

07-Apr-2017

L.Petrykowski

- Consequences of injury
- Continuing entitlement

No Summary Available

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- WSIA

Other Case Reference

- [w2317n]

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WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 293/17

BEFORE:

L. Petrykowski: Vice-Chair

HEARING:

January 30, 2017 at Toronto
Oral

DATE OF DECISION:

April 7, 2017

NEUTRAL CITATION:

2017 ONWSIAT 1044

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer decision dated
November 21, 2013

APPEARANCES:

For the worker:

M. Russell, Lawyer

For the employer:

C. Gibbons, Lawyer

Interpreter:

N/A

**Workplace Safety and Insurance
Appeals Tribunal**

505 University Avenue 7th Floor
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**Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail**

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REASONS

(i) Introduction to the appeal proceedings

- [1] The worker appeals a decision of the Appeals Resolution Officer (“ARO”) of the Workplace Safety and Insurance Board (“Board”), dated November 21, 2013, which determined that the worker did not have ongoing entitlement for bilateral carpal tunnel syndrome (“CTS”) and no entitlement for right CTS surgery under Claim A (accident date: March 18, 2003). The ARO also determined that the worker’s right wrist strain under Claim B (accident date: January 18, 2011) had resolved and that there was no right CTS entitlement under that claim. Finally, the ARO determined that the worker did not have bilateral trigger fingers entitlement under Claim A. That decision was rendered following an oral appeal hearing at the Board’s Appeals Branch.
- [2] The worker attended the Tribunal hearing with her representative, Mr. Russell, a lawyer. The employer attended the Tribunal hearing with its representative, Ms. Gibbons, a lawyer.

(ii) Issues

- [3] The issues to be decided in this appeal, as similarly identified on the Tribunal’s hearing ready letter dated June 12, 2015 and agreed to by the parties, were as follows:
1. Whether the worker has ongoing entitlement for bilateral CTS under Claim A, including entitlement for right CTS surgery?
 2. Whether the worker has entitlement for right CTS under Claim B?
 3. Whether the worker has entitlement for bilateral trigger fingers under Claim A?

(iii) Background

- [4] I have reviewed the entirety of the documentary record and the following background facts are briefly noted.
- [5] The worker, now age 60, began working with the accident employer around 2002. Her claim was established with the Board for a disablement injury dated March 18, 2003. This represents Claim A in the Tribunal proceedings. The worker reported an onset of bilateral wrist discomfort, which she related to job duties associated with court support activities. The Board’s operating level accepted claim entitlement for bilateral CTS.
- [6] The worker was granted loss of earnings (“LOE”) benefits from July 11, 2003 to July 23, 2003. She also received partial LOE benefits from July 23, 2003 to September 2003 while participating in modified duties. A recurrence was accepted in 2004 and 2005. There was no further reporting in this file and a ruling was never made on the worker’s entitlement for a permanent impairment or non-economic loss (“NEL”) assessment.
- [7] The worker had another accident at work on January 18, 2011, when she fell in a parking lot. It was accepted that the worker sustained multiple soft tissue injuries, including to the low back, right hand/arm, right hip, knee and neck. This represents Claim B in the Tribunal proceedings. The Board’s operating level accepted claim entitlement for her various injuries. The

worker continued to report symptoms related to her right hand/wrist. The worker then underwent right CTS surgery on November 4, 2011.

[8] The worker received full LOE benefits under Claim B from January 19, 2011 to February 10, 2011, and then partial LOE benefits from February 10, 2011 to May 17, 2011. She was also paid full LOE benefits for May 17, 2011. It was then concluded by the Board's operating level that the worker's injuries had fully resolved and there was no ongoing entitlement, including entitlement for CTS and surgery.

[9] The worker claimed ongoing entitlement for her CTS condition and entitlement for bilateral trigger fingers under Claim A. The Board's operating level concluded that there was no ongoing entitlement for the CTS, including for surgery. Additionally, the Board's operating level denied entitlement for the bilateral fingers under Claim A.

[10] The worker's representative objected to the above-described decisions and the matter was referred to the Board's Appeals Branch. The determinations of the Board's operating level were upheld by the ARO in the decision dated November 21, 2013 which included the following conclusions:

CONCLUSIONS

1. I find that the worker does not have ongoing entitlement for bilateral CTS, and in particular, she does not have entitlement for right CTS surgery.
2. I find that any right wrist strain sustained as a result in the January 18, 2011 accident has fully resolved and there is no entitlement under the January 18, 2011 claim for right CTS.
3. There is no entitlement for the bilateral trigger fingers as related to the worker's general job duties disablement injury of March 18, 2003.

[11] The correctness of the above-noted ARO determinations, only insofar as they have been above identified in the issue agenda for this appeal, is the matter now before the Tribunal.

(iv) Law and policy

[12] With respect to both Claims A and B, the *Workplace Safety and Insurance Act, 1997* (the "WSIA") is applicable to this appeal. In determining benefit appeals, the Tribunal is required to apply Board policy in accordance with the provisions of section 126 of the WSIA.

[13] Subsection 13(1) of the WSIA states: "A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan".

[14] Subsection 2(1) of the WSIA defines "impairment" as a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss.

[15] Subsection 2(1) of the WSIA defines "permanent impairment" as an impairment that continues to exist after a worker reaches maximum medical recovery.

[16] Subsection 46(1) of the WSIA provides that if a worker's injury results in permanent impairment, the worker is entitled to compensation for non-economic loss.

[17] Pursuant to section 126 of the WSIA, the Board stated that the following policy packages: 38, 223, 224, 226, 241, 261, and 300 - Revision #9, would apply to the subject matter of this appeal.

[18] I have considered these policies as necessary in deciding the issues in this appeal, in particular:

- *Operational Policy Manual* (“OPM”) Document #18-05-03, “Determining the Degree of Permanent Impairment”;
- OPM Document No. 11-01-05, “Determining Maximum Medical Recovery (MMR)”;
- OPM Document No. 11-01-01, “Adjudicative Process”;
- OPM Document No. 18-03-02, “Payment and Reviewing LOE Benefits (Prior to Final Review)”;
- and
- OPM Document No. 15-03-01, “Recurrences”.

(v) The worker’s testimony

[19] The worker testified that she was employed with the employer since 2002. She remained there until she retired in 2015. She was the assistant coordinator in court services, in an administrative office. She worked with other court services before that. There was a government downloading of provincial offences in 2002. She has seen the job functions document from 2003 and it reflects her job responsibilities. She was on the computer up to five hours per day. She was in the business support section and oversaw staff there. She keyed in information, worked on backlogs and statistics, and backed up the counter where ten cashiers were present. She also did bank deposits. They were busy and doing court-related stuff. She sometimes had to go repetitively through files for 2 – 3 hours at a time. There was lots of mousing involved in her court assistant role. There were template screens that she populated. She did lots of investigating and switched back and forth through screens.

[20] The worker then stated that she had a WSIB claim in 2003. She had pain in both hands but more on the right. She had trigger finger affecting her right thumb. She had constant throbbing pain and numbness. This was associated with keyboarding, flipping documents, and manual counting of money. She opened/closed the court on many occasions. She did gripping when pulling information out and matching them to files. There were lots of notice of trials to go through. Her hand would hurt from this gripping. Her condition got worse as the week went on over five days of work. She would rest them at home and put hot water on them at work. She had medications for her hands. Her hands still hurt over the weekend. This affected her mopping. It was a bit better over the weekend because of the rest.

[21] The worker further testified that she sought medical attention all the time for this. She saw her family doctor, Dr. Katz, who referred her to Dr. Handelsman. Dr. Katz’s notes from 2005 to 2012 are in the documentary record. She saw Dr. Handelsman in 2003 and saw him before Dr. McKee. She received cortisone for her thumbs from Dr. Handelsman two times. She then had right thumb surgery in 2005 for the trigger condition. She must have missed time in 2003 but cannot recall. She missed time for the surgery. She cannot recall about lost time from her recurrences. Her carpal tunnel syndrome did not resolve and it never went away. She was doing full duties in 2005. It was difficult. She had pain but not the trigger problem.

[22] The worker then stated there was a 2005 claim for a chair-related injury. Her chair broke and she held herself against the wall not to fall. There was lots of pressure on her hands at the time. She twisted so as not to fall and injured her back. She had more pain in her hands from this incident. S. was her manager after the 2005 incident to 2011. The worker had other supervisors as well, including C., T., A., A., and finally K. She spoke to her supervisors about her hands and that they were hurting. There was no specific time in this regard after 2005 but she must have mentioned it. All supervisors knew she had hand problems. S. was on the other side of the building and some days she did not see her. S. did not work regularly with her but rather did deposit checks. The worker rotated and stayed in administration and then went to trial scheduling which was in the same office. She moved close to S. in 2010 who was behind a wall but never worked beside her. She spoke with S. in 2011 about her modified duties. She does not remember speaking about this matter with S. before her 2011 fall.

[23] The worker then explained that her change of duties in 2010 involved reviewing 800 – 1000 invoices per month for proper payment. She also did gripping in court document verification. She handled invoices and looked for information. She put documents in alphabetical order and this was repetitive. This change was maybe a bit better because there was less computer work but she was always on the computer. She populated fields with information and worked with Excel. She also used a calculator especially at the end of the month. She had symptoms from this, including pain in her hand. This was usually on the right side.

[24] The worker also stated that she had a horrendous fall in January 2011. She slipped on ice in front of her building. She struck the ground and the wind was knocked out of her. She mostly fell on her left side and her attire was ripped. She tried to flip herself and could not get up with her hands. She was helped by a prosecutor and then went to see S. She was right-handed and this incident impacted her hands. She kept slipping when trying to get up. She completed the Form 6 in the documentary record and noted various injuries there. She lost time from work until February. She then went on modified work and was re-assessed by the health department there. Dr. Katz completed functional forms for her. She remained on modified work in 2011 until she left. This involved not doing invoice calculations. People would put tabs for calculations. It was otherwise the same job. Her keyboarding was limited and she had a special mouse.

[25] The worker then stated that her hand symptoms got worse after the fall. Her pain and carpal tunnel condition worsened. She could not take minutes of the meeting. Her hands were numb and she took medications. She then had surgery. She had splints before 2011 and now uses them occasionally. She left work, managed her pain, and cannot do repetitive stuff. She had carpal tunnel surgery in November 2011. She had two trigger finger releases on the left side in 2012 and one on the right in November 2012. She then had another one on the right hand in March 2015. Dr. McKee recommended new splints. The carpal tunnel surgery helped with numbness but she still had pain. She left in 2015 due to pension issues. She had to go to the province for an amalgamated pension but left work for pension reasons. There were no outside activities contributed to her carpal tunnel condition after 2003. She limited her activities at home.

[26] The worker also testified she moved close to S. in 2010 who might have asked about the worker's hands but she cannot recall. There were absences after 2010. It was not 50% of the time from 2010 onwards. It was less than that. She cannot recall about her knee injury. With respect to her fall, she landed on her backside and tried to get up on her right side using her hands. Her right elbow was affected in the fall and there was ice everywhere. There was another fall in July 2012 in a grocery store. She slipped on grapes there and did not have a cart. She injured her

knees in going down. She fell forward and tried to use her left hand in that fall. She fell mostly on her knees. Her hands hurt at that time but there was no more pain in her hands because of that fall. She came back to ½ days (3 hours daily) after being off work.

[27] The worker further testified that her friends and mother helped her makes cookies around Christmas for the office. She stopped this after her carpal tunnel surgery and cannot remember when. She always had help with the cookies. She also had a cookie mixer. It just came out and had to be cut and baked. With respect to the clinical records, she stated that she saw the doctor around May 5, 2009 and had physiotherapy on/off for a long time. She cannot recall when that started but maybe after 2005 for both the trigger and carpal tunnel conditions. She had modified work from 2010 onwards. This helped her condition because there was less hand use and less repetition. She has no thyroid problems. She takes no diabetes medication. She is 60 years old. She took Meloxicam a long time ago. She took Advil all the time and Tylenol. She used wax and hot water. She took an anti-inflammatory in the past but cannot recall the name. She was 150/155 pounds in 2003 and 170/175 in 2011. She had knee surgery in 2010. She had no other injuries from 2003 to the present.

(vi) The testimony of the employer's witness, S.

[28] S. testified that she is the manager of court services. She previously oversaw provincial offences court and a staff of 60 people. She was the worker's manager. She worked with court services for 15 years. The worker reported to her supervisor. The worker reported to S. if there was no supervisor. S. was familiar with the worker's job duties. S. worked her way up and did that work herself in the past. She met with the worker and her team. They went over procedures and training. She knew the worker's job from start to finish. She had status meeting with the worker every month and daily interaction. Court services provided administrative support to the provincial offences court and the hearing rooms.

[29] S. then stated that the worker's job was 35 hours per week. She was a support assistant A. between 2003 and 2011. S. came in 2005 and the worker was in trial scheduling in 2009. She then became an interpretation lead. The worker's duties in trial scheduling include preparing dockets, case transfers, trial/resolution scheduling, statistical reports, liaising with stakeholders and other leads, and investigating information. They have file folders that are 4 – 5 inches wide with tiers therein. A worker had to look for court history and print out screens from pulled folders and look for missing ones. Trays would be pulled from cabinets and this involved standing in front of the cabinets. It was typically done twice per week, due to staff misfiling, but sometimes not at all each week. There was some telephone work in the trial unit. There was computer work to add cases, search databases, run monthly stats, and investigate things. A lot of the work was talking to other staff, doing orientations, which the worker also did sometimes, and ergonomic checklists. The work involved two hours per day of computer use but it was not a data entry job. There was lots of variety in the tasks. The worker was in this position from 2005 to 2009. She did this intermittently before 2002/2003.

[30] S. then testified that the worker ceased that work in October 2009 and went to the interpretation unit. She was placed there due to rotation and had seniority. This involved monitoring staff, coordinating interpreters, updating court documents, balancing dockets, and monitoring weekly lists, and looking for real-time captionist request. It also involved reconciling invoices for accuracy. Support Assistant B was later added to the reconciliation work for invoices. The invoices were re-designed for easier reading. A batch program of 300-400/month

was introduced. Support Assistant B was added in April/May 2010. The database was developed in 2009 and implemented in April 2010 after training. Support Assistant B took some work from Support Assistant A but both A and B were involved with invoices in 2010. Batch slips were done in 2012 and fewer signatures were required then. 300-400 invoices were done monthly but this dropped over time. It is currently 300 per month but was 500 per month at the busiest time in 2010/2011 and then declined.

[31] S. went on to explain that the worker's work in the interpretation unit involved being responsible for checking databases, interpreter tables, and weekly lists. It also involved a little bit of e-mail and statistical reports. Support Assistant B did verification reports in 2010, which was formerly done by the lead. It became more computerized with Support Assistant B. The worker needed to check the computer for problem-solving. There was very little data-entry in the lead position. A job demands assessment was conducted on keyboarding and mouse around that time. The former was fairly low at one to one and a half hours per day. The lead had autonomy in doing the work and there was lots of variety. This was fairly consistent work over the years but some things changed. The lead interacted with staff, answered questions, and checked workflows. The worker resolved interpretation issues and worked with paper files including balance sheets and the adding machine. Invoices were reconciled by staff and double checked. The worker sat around the corner from her.

[32] S. also testified that the worker had to investigate older invoices and go to the lower level room. Someone else would carry the boxes in this regard. It was very infrequent to pull old files, at one to two times per year. The mouse was used in the interpretation unit and there was a change after the ergonomic consultation. The worker had a larger optical mouse and a roller mouse was bought for her after the ergonomist was involved in 2013. The worker's hand only had to move lightly for it to work. The interpretation management system was implemented in 2010 with lots of training before that. This eliminated 85% of data entry for the worker and automated stuff. Less paper came in and request forms became electronic. This resulted in a 90-95% reduction in papers. Support Assistant B added quality control and took some statistical/review work and invoices away from Support Assistant A.

[33] S. further stated that she had a discussion with the worker about her hand problems. The earliest one was shortly after the incident when the chair gave way and the worker was injured. The worker was off and got restrictions for her hands. S. met with the worker and others and went over the accommodated work plan in 2005. The worker had modified work in July and August. They talked about whether she could pull out files. S. sent a detailed letter for the worker's health care provider. The worker had trigger thumb problems over the years. They discussed in 2011/2012 that the worker was off for many days. They talked about aging of the hand but not specifics about work. The worker made lots of Christmas cookies and gave them as gifts. The worker noted she was getting to old to make the cookies and had sore hands. This was around Christmas 2010.

[34] S. then went on to testify that there were no discussions about between 2005 and 2010 about decreasing repetitive duties. The supervisors did not report that the worker had hand problems to her over this period. She met regularly with them and safety issues would have been discussed, including accommodation. There was ergonomic involvement and she reviewed checklists. She ensured staff were trained and reviewed this with workers. Supervisors handed these documents to her. She was familiar with the worker's attendance and accommodations. The worker was off work for a while in 2005 due to the chair-related injury and came back on

modified work. She was off for a few days in December. There was accommodation for the worker in 2005 and a gradual return to work. The worker was in the call-centre for “light duties” to reduce hands involvement. The restrictions were bending, lifting for the back injury, and repetitive use of the hands. The worker’s attendance was fairly good until surgery in 2006, and had limited walking for a short time after she came back. She had good attendance in 2007 to 2009 and no accommodations then.

[35] S. further stated that the worker had an accident at home in 2010 on her stairs when she hurt her knee. She lost over 100 days of time. She returned to accommodated work in June 2010 and a gradual return to her hours with no limitations. Others would deliver files to her. She had accommodations for lifting/carrying and would take mini-rests. There was a restriction on repetitive use of the hands in 2010 and someone else retrieved folders for her. This accommodation lasted until August/September. She then had full duties until the end of the year. She had restrictions in 2011 following her fall. This involved stooping, bending, carrying, and no repetitive hand motions. She checked the numbers were correct and Support Assistant B took on some more work, plus the supervisor, in balancing dockets. The worker was accommodated with modified work until her 2015 retirement. The worker attended work 50% of the time between 2010 and 2015. She had lengthy absences and bad health.

[36] S. also stated that she saw the worker daily when she was doing Support Assistant A work but she did not sit with her. S. had personal knowledge of the job and there was a job demands analysis. S. only saw reports of computer activity. She knew the volume of the key strokes and deduced the time of computer work from this. There was an email from the ergonomist in 2005 where an optical mouse was recommended and obtained for the worker. She wrote a letter dated June 8, 2005 to the worker who returned to light duties. There was another letter dated July 11, 2005 after a meeting with D. This involved more detail and involved the health care provider. There was a suggestion to alternate duties then. There were a variety of tasks to do in the lead job. All of these tasks involve manual clerical duties and not just involving the hands. This dealt with back restrictions and did not deal with leadership and verbal tasks. The letter dealt with specific accommodation. There was an email dated July 28, 2005 about the worker’s return to modified work and whether it worked for her. The modification came to an end and the worker returned to regular duties for years. K. was the worker’s supervisor in March 2012 who reported to S. They implemented Dr. McKee’s recommendations then. The worker’s later permanent restrictions resembled those in the email but not all modifications were noted therein.

(vii) Submissions of the worker’s representative

[37] On behalf of the worker, Mr. Russell submitted that the CTS and recurrences was initially granted and is not being revisited in these proceedings. The WSIB accepted a disablement from job duties. The employer’s witness testified about what she believed the worker did, based on experience, knowledge and job demands, and not based on direct examination. The worker testified about her actual duties and her evidence should be preferred. The job description of Support Assistant A showed the manual clerical functions and regular use of computer for the worker, up to the change in job function. Many duties required the use of her hands within a small number of tasks. The worker testified about her activities and how they aggravated her hand symptoms. There was rest on the weekends and the worker self-modified. Her condition got worse after 2005 and she was trying to do a regular job. The worker persevered with pain

throughout. She testified her supervisors knew of her pain. The worker had a wrist splint and was on medication. She saw her health care providers intermittently. The ARO noted 12 visits between 2005 and 2011. The worker testified that her pain significantly increased after the fall. She was then referred to Dr. McKee and need surgery for CTS. Dr. McKee's report dated December 10, 2012 is relevant and supports the worker's appeal. Significant restrictions were put in place following the 2010 surgery. Dr. Katz's clinical notes plus the worker's testimony support medical continuity. The worker was back in physiotherapy in May 2009 for CTS and there was no other incident/accident that caused this. Dr. Freiberg's report dated September 16, 2004 noted the worker needs restrictions from keyboard use and gripping. Ongoing entitlement for CTS from the 2003 claim should be allowed and returned to the Board for benefits nature/duration. With respect to the 2011 matter, the relevance falls away as if the first issue is allowed. The CTS was already there as the worker testified. The symptoms were aggravated by the fall. If the 2003 claim is denied, it should be allowed under the 2011 claim as an aggravation. With respect to the trigger fingers, this is all bound together with the work duties. The same constellation of risk factors from work relate to both CTS and trigger fingers. If allowed, this can be referred to the Board for nature/duration of benefits. The worker returned from knee surgery in June 2010 and had further medical appointments for her work-related conditions. Dr. Somerville's report is flawed and is at odds with what is already adjudicated in the file, as CTS was already allowed.

(viii) Submissions of the employer's representative

[38] On behalf of the employer, Ms. Gibbons submitted that the 2011 claim was allowed for soft tissue injuries that resolved. There was no relationship between CTS and the accident/injuries in 2011. There was no significant injury to the right hand. The Medical Discussion Paper by Dr. Graham supports that a minor injury would have unlikely caused the worker's CTS in this case. The fall was not compatible beyond a minor strain. The problems with the CTS and hands pre-dated the fall in 2011. The worker had been off work for an unrelated issue and had arthritis. There was no worsening in the 2011 fall and that matter should be denied. There is also no link between trigger fingers and the 2003 claim. The work duties were not involved. Dr. McKee's report talks about factories but the worker did not do factory-type work. The medical evidence is not persuasive in the worker's appeal, both for CTS or trigger fingers. The worker did not have a high risk for trigger finger development as she was not gripping tools for prolonged periods, nor was forceful and repetitive action involved. There is no evidence that trigger fingers resulted from work or CTS. The worker's appeal should be denied in that regard. With respect to the 2003 claim, medical continuity thereafter has not been established based on the medical evidence nor is there clinical compatibility as related to potential recurrence entitlement. The worker did not provide direct evidence about problems to her supervisors and no other parties confirmed it. She had no work restrictions between 2005 and 2010 and was doing regular work in the interim. There are significant gaps in Dr. Katz's records. There is not specific attribution to work in those records. There is no CTS in the notes after 2005 until 2009, and the worker was affected by other problem such as her knee. Ongoing entitlement should not be allowed as there is insufficient evidence. There was no significant contribution from work. The worker was then absent from work 50% of the time from 2010 and her condition worsened. The worker never worked six hours per day on the computer. Dr. Somerville's report dated May 16, 2012 does not support the worker's CTS or trigger fingers appeal. There was a lack of risk factors in the work, similar to Dr. Graham's explanation in the discussion paper. The

worker got worse from less work and this does not support causation. There are lots of unknown causes for this condition. Dr. McKee did not look at the worker's job duties. The worker's appeal should be denied.

(ix) Analysis and conclusions

1. Whether the worker has ongoing entitlement for bilateral CTS under Claim A, including entitlement for right CTS surgery?

[39] The first issue under appeal pertains to whether the worker should have ongoing entitlement under her claim, for areas of injury (bilateral CTS) that the Board's operating level accepted as compensable flowing from Claim A (accident date: March 18, 2003). The scope of ongoing entitlement also includes the issue of whether the worker should have entitlement for right CTS surgery that was carried out on November 4, 2011. I note that the Board's operating level accepted subsequent recurrences as compensable in 2004 and 2005 under the worker's claim. The correctness of those determinations is not at issue in the present appeal.

[40] In that context, I have considered that OPM Document #15-03-01 entitled "Recurrences" states that "to identify a recurrence, the WSIB must confirm that there is clinical compatibility between the original injury or disease and the current condition, or a combination of clinical compatibility and continuity". That Board policy goes on to note that clinical compatibility needs to be established between the worker's current condition and that which followed the initial accident. The Board policy goes on to note that continuity ("a connection between the original clinical condition and the most recent problem or problems") also needs to be considered. The standard of proof required in Tribunal proceedings is proof on the balance of probabilities.

[41] I have also considered that the WSIA provides a definition of permanent impairment in subsection 2(1) stating "impairment" means a physical or functional abnormality or loss (including disfigurement) which results from an injury and any psychological damage arising from the abnormality or loss while "permanent impairment" means impairment that continues to exist after the worker reaches maximum medical recovery. Subsection 46(1) of the WSIA sets out entitlement in instances where the worker is found to have a permanent impairment stating "if a worker's injury results in permanent impairment, the worker is entitled to compensation under this section for his or her non-economic loss".

[42] I have next considered that OPM Document #11-01-05, entitled "Determining Maximum Medical Recovery (MMR)", states that:

...Workers reach maximum medical recovery (MMR) when they have reached a plateau in their recovery and it is not likely that there will be any further significant improvement in their medical impairment.

[43] In this vein, I have finally considered that OPM Document #11-01-05, sets out factors which are to be considered when determining whether or not a worker has reached MMR, as follows:

Identifying MMR

If a worker has a permanent disability/impairment, decision-makers identify when MMR is reached and record the date. A worker may have reached MMR and still be receiving treatment, such as physiotherapy or drugs where a significant improvement is unlikely.

Decision-makers determine when MMR is reached based on the following information which includes but is not limited to

- clinical reports from the treating health professional(s)
- specialists' report(s), where appropriate
- reports from agency(ies) providing treatment and/or evaluation, (e.g., Regional Evaluation Centres)
- information from the worker on his/her medical impairment
- external, evidence-based medical/scientific guidelines on disease and injury-specific impairment and treatment,
- the opinion of WSIB clinical staff, if obtained.

[44] In considering the evidence about the issue in dispute, I note that the worker's compensable bilateral CTS was allowed on the basis that a disablement injury occurred around March 18, 2003 associated with her workplace duties. The preponderance of medical evidence supports the existence of the worker's bilateral carpal tunnel syndrome between 2003 and 2004, with a greater degree of clinical severity on the right-side. A concomitant trigger finger pathology also affected the worker around this time, but this is not the same diagnosis or condition as carpal tunnel pathology. Electromyography ("EMG") testing of the worker's upper extremities in 2004 was "normal" as noted by Dr. Handelsman, a rheumatologist. That same doctor later assessed the worker on March 3, 2005 and noted the existence of an "element CTS", which particularly affected the worker's right hand. Dr. Handelsman then noted on December 14, 2007 that there has a "possible element of [right] CTS". On February 17, 2011, the worker saw Dr. Vincent, an orthopedic specialist, who noted the worker had "bilateral numbness in her hands" and "likely carpal tunnel syndrome" alongside stenosing tenosynovitis affecting her fingers. EMG testing was carried out on March 30, 2011 which stated "history suggested bilateral carpal tunnel syndrome" but the nerve conduction studies only showed a mild degree of neuropathy on the right side. Dr. Weinberg, an orthopedic specialist, assessed the worker on May 17, 2011 noting she had bilateral carpal tunnel syndrome, which was a pre-existing condition "aggravated by the recent slip and fall". The slip/fall involves Claim B, which is further discussed later in this decision.

[45] I have also considered the clinical records from Dr. Katz, family physician, over the 2005 to 2010 period, before the establishment of Claim B in January 2011. The worker was seen dozens of times before the establishment of Claim B. I find it significant that the worker was not seen on a regular basis by her family physician over this period for carpal tunnel symptomology/pathology. The medical evidence does not support its existence over this period as an ongoing or clinically significant condition. However, the worker had experienced numerous non-compensable conditions over this period affecting multiple body parts. Those, rather than bilateral CTS, were the mainstay of her clinical treatment over this period. The paucity of medical evidence over this period weighs heavily against allowing ongoing entitlement in the worker's claim for bilateral CTS related to Claim A originating in 2003. The worker had many clinical opportunities for Dr. Katz to address and potential ongoing CTS-related pathology but the preponderance of medical evidence did not support this as an ongoing clinical concern over this extensive period of time.

[46] I have also considered that Dr. Mezei, a rehabilitation clinical director, noted in correspondence dated January 20, 2012 that the worker had been treated in the past for "an aggravation of her pre-existing hand/wrist and carpal tunnel injuries due to a fall from her chair". This was related to a claim originating on February 15, 2005 (Claim.....924), which is not

jurisdictionally before me in the present Tribunal appeal. The existence of that claim further suggests that the worker's claimed carpal tunnel problems were remote and distinct from Claim A in 2003 when viewed from a temporal and clinical perspective.

[47] I have further considered that following 2005, the worker returned to her regular duties which were of an administrative/clerical nature and she continued to do those duties for several years without medical accommodation related to any claimed bilateral CTS. Evidence that the worker's employer was aware of any ongoing bilateral CTS condition over these years is also largely absent, as supported by the employer's witness whose testimony at the Tribunal hearing I found to be credible, reliable, and straightforward. The worker also did not communicate with the Board over this period of time in relation to any ongoing bilateral CTS. All of these factors do not support that there is any basis to support ongoing entitlement, including recognition of a permanent impairment, for a claimed bilateral CTS associated with Claim A. I am cognizant that Dr. Handelsman noted on December 14, 2007 that there was a "possible" right-sided element of CTS in the worker's multi-faceted upper extremity complaints, but this was only a possible diagnosis, and was unilateral in scope (not bilateral). This piece of evidence is simply insufficient to support ongoing bilateral CTS associated with Claim A.

[48] On the issue of clinical compatibility between the worker's injury in 2003 under Claim A and the right CTS surgery that was carried out on November 4, 2011, the Board obtained a medical opinion from Dr. Somerville, a specialist in occupational medicine, on May 16, 2012. Dr. Somerville did not support any relationship between Claim A and the worker's later claim for ongoing entitlement, including right CTS surgery, as follows:

Following a review of the file documentation, the injured worker's CTS (carpal tunnel syndrome) of the right wrist requiring release surgery on November 4 2011 is not consistent with work duties as described in Memo # 36, Memo # 37, Memo #39, and elsewhere in file. About 50% of cases of CTS have no discernible cause and are absent the known risk factors for this condition. Medical risk factors include diabetes, wrist fracture, obesity, connective tissue disease, hypothyroidism, pregnancy and others. (Of note, Multidisciplinary Healthcare Assessment of April 17 2011 states that the injured worker is five feet two inches tall and 185 pounds.) Ergonomic risk factors are more poorly understood, but include forceful repetitive wrist movements and extensive use of vibratory equipment. Jobs with the highest rates of CTS are those associated with high levels of hand-arm vibration, high requirements for hand force, prolonged work with a flexed or extended wrist, and high repetitiveness, seen in such industries as meat and fish processing, forestry and assembly line work. The risk of CIS among keyboard users is relatively low. The injured worker's work duties did not involve evidence of exposure to risk factors known to contribute to CTS. Further, she has had significant periods of time off work since the accepted accident of March 18 2003 as well as periods of modified work duties, however her hand symptoms recently worsened. For above reasons the CTS surgery of November 4 2011 is not related to the original injury.

[49] In my view, the medical evidence as a whole, including Dr. Somerville's explicit above-reproduced opinion did not support that the worker's original injury under Claim A was compatible with the right CTS surgery that occurred eight years later. However, I have later addressed in this decision how the compensable fall under Claim B (not Claim A) was significantly related to the worker's right CTS condition as an aggravation.

[50] Therefore, having reviewed the totality of evidence before me, I find on a balance of probabilities for the aforementioned reasons that there is no basis to allow ongoing entitlement, including recognition of a permanent impairment or entitlement to right CTS surgery, for the worker's bilateral CTS under Claim A. The worker's appeal is denied in this regard.

2. Whether the worker has entitlement for right CTS under Claim B?

[51] I have earlier determined that there was no basis to allow ongoing entitlement under the worker's claim for bilateral CTS as related to Claim A. In the second issue under appeal, I have considered that the worker sustained a significant and traumatic fall on January 18, 2011, which had a significant impact on her right-sided carpal tunnel pathology under Claim B. The worker's testimony at the Tribunal hearing described this as a "horrendous" fall suggesting something far more than a minor or trivial incident.

[52] From a temporal and clinical perspective, I find that there was a substantial aggravation of the worker's right hand/wrist condition following the *compensable* fall on January 18, 2011 under Claim B. However, this appreciates the fact that the evidence as a whole suggested that the worker had some degree of pre-existing carpal tunnel pathology immediately prior to the January 18, 2011 incident.

[53] In that light, I have considered that the worker spoke with the Board on June 8, 2011 (Board Memorandum #9) where she explained that her fall was on the right side "and she feels she reinjured her wrists while trying to get up". The Worker's Report of Injury (Form 6) dated January 20, 2011 noted bilateral hand/wrist/finger injuries but her written description in the "Additional Information" section of that form noted that her "right arm + hand" were affected in the incident. The Employer's Report of Injury (Form 7) dated January 21, 2011 only documented a right hand (not left hand) injury. The initial medical report (Form 8) completed by Dr. Mezei on January 20, 2011 suggested that the worker hurt her right wrist/hand/fingers (not her left wrist/hand/fingers) after slipping on ice and falling outside the building where she worked. That doctor was aware of a "pre-existing hands" condition on that occasion but was not more specific than that.

[54] In my view, the contemporaneous descriptions of the accident suggested that a unilateral (right-sided) transmission of force particularly affected the worker's right hand/wrist in this compensable accident. The medical evidence following that time also suggested an ongoing right hand/wrist-related problem affecting the worker, to a degree that did *not* exist before January 18, 2011. Dr. Vincent suggested that further EMG studies be carried out after consulting with the worker on February 19, 2011. As noted earlier, EMG testing was carried out on March 30, 2011 which noted "history suggested bilateral carpal tunnel syndrome" but the nerve conduction studies only showed a mild degree of neuropathy on the right side. I find this to be objective electrodiagnostic evidence suggesting that the worker's right-side (not left side) had been affected by the fall that occurred two months earlier. These findings were different and worse than earlier EMG testing that occurred in relation to Claim A, as discussed earlier.

[55] I have next considered that Dr. Mezei had been participating in the worker's rehabilitation since January 2011 and noted that the worker had an injury to her right hand, which was "painful in pronation and supination and radial flexion", as noted in his report dated April 7, 2011. Dr. Mezei then noted on April 11, 2011, in a treatment extension request for the Board, that the worker's hands had improved 20% in strength and required a surgical consultation. I find it significant that a surgical consultation was never suggested for the

worker's right hand or carpal tunnel condition prior to the fall under Claim B. This temporally and clinically suggests that the fall under Claim B was a significant contributing factor in the worker's need for surgical assessment and later intervention. No other factor or event can be seen as a significant intervening factor in that regard. The worker was then seen by Dr. Weinberg on May 17, 2011, as noted earlier. Dr. Weinberg performed a physical examination, was familiar with the worker's medical/work history, and noted she had bilateral carpal tunnel syndrome, worse on the right, which was a pre-existing condition "aggravated by the recent slip and fall". The worker later saw Dr. McKee, an orthopedic surgeon, on July 5, 2011 who noted the worker's "troubles were aggravated by a fall at work and things have not been back to normal ever since that". Finally, Dr. Mezei's correspondence dated January 20, 2012 noted that the worker was "treated for a slip and fall on January 18, 2011, which further aggravated her condition resulting in carpal tunnel surgery on November 4, 2011... under Claim [B]".

[56] On the whole, the evidence supports that the worker sustained a right-sided aggravation of her pre-existing carpal tunnel condition in the compensable accident of January 18, 2011 under Claim B. This aggravation was so substantial that multiple health care providers documented its existence and its temporal relationship to the events of January 18, 2011. Electrodiagnostic testing of the right median nerve also revealed a worsening in nerve conduction on the right-side, which correlates to the compensable accident of January 18, 2011, particularly affecting the right-side. Similarly, Dr. McKee noted on July 5, 2011 "things have not been back to normal ever since" that fall. I also find it significant that this was entirely consistent with the worker's testimony at the Tribunal hearing when she testified "her hand symptoms got worse after the fall. Her pain and carpal tunnel condition worsened".

[57] Therefore, having reviewed the totality of evidence about this second issue, I find on a balance of probabilities that the worker has established claim entitlement under Claim B for an aggravation of her right-sided carpal tunnel syndrome. This includes the fact that the worker required surgery for this condition on November 4, 2011, which I find to be clinically compatible with the mechanism of injury under Claim B, given the above-noted preponderance of medical evidence which undergirds such a conclusion. The compensable accident under Claim B made a significant contribution in the need for this surgical intervention. The worker's appeal is allowed in this regard. The nature and duration of any additional benefits that the worker may be entitled as a result of these determinations under Claim B is remitted to the Board for future adjudication, subject to the usual rights of appeal.

3. Whether the worker has entitlement for bilateral trigger fingers under Claim A?

[58] I have finally considered the issue of potential claim entitlement for bilateral trigger fingers under Claim A. That claim was allowed as a disablement in 2003 related to the worker's bilateral CTS and job duties. The worker requested that entitlement be expanded under Claim A to include bilateral trigger fingers, which she has similarly related to her job duties as a disablement.

[59] The worker's appeal has been largely advanced on the basis that the onset of the trigger fingers was due to an accident in the nature of a disablement at work that should be compensated for. I am required to apply Board policy in the adjudication of this appeal as a result of the applicable provisions of section 126 of the WSIA. OPM Document No. 15-02-01, "Definition of

an Accident”, indicates the definition of “disablement” to include “a condition that emerges gradually over time” and “an unexpected result of working duties”.

[60] In order to establish that an accident in the nature of a disablement occurred, proof of accident is required. The standard of proof required in Tribunal proceedings is proof on the balance of probabilities. Tribunal caselaw has held that in disablement claims the presumption does not apply. In a claim for an accident in the nature of a disablement, there may be many factors contributing to the impairment but it must be shown on a balance of probabilities that the workplace factors played a “significant contributing” role in the cause of the impairment. See Tribunal *Decision Nos. 1672/04; 1386/03; 426/07; and 583/07*.

[61] It is well established in the Tribunal’s jurisprudence that the appropriate test for entitlement is whether the compensable work-related factors which are alleged to be related to the condition can be demonstrated, on a balance of probabilities, to have contributed significantly to the development of the condition. The fact that other non-compensable factors may also have contributed significantly to the condition will not affect entitlement.

[62] In this case, to be entitled to benefits for a condition diagnosed as “trigger fingers” (or stenosing tenosynovitis) over the years, the worker need not demonstrate that the work-related factors were the sole contributing factor to the onset of the claimed condition. It is enough to show that the work-related factors contributed significantly, regardless of the existence of other non-compensable factors which might also have contributed significantly.

[63] In this light, the leading Tribunal decision, *Decision No. 915*, 7 W.C.A.T.R. 1 at 134, states in part:

The Accidental Injury Need only be One of the Significant Contributing Factors

It is also a first principle of personal-injury, civil-litigation law that a defendant's negligence does not need to be the sole cause of the damages claimed. It is equally well established in workers' compensation law that for compensation entitlement to arise, the disability does not have to result solely from the accidental injury.

In its decisions to date as to the meaning of results from, the Appeals Tribunal has taken the view that a disability may be said to have resulted from an injury if the injury made a significant contribution to the development of the disability. The Panel agrees with that interpretation.

This is almost exactly the same place the courts have reached in the modern development of the civil-litigation concept of cause in personal injury cases. The definition of cause in the courts has had a long and checkered history but there is general acceptance of the proposition that consequential damages will be found to have been caused by a defendant's negligence if that negligence made "a material contribution" to the damage...

This Panel is, therefore, satisfied that a disability must be seen to have resulted from the compensable injury (and, therefore, to be compensable) if the injury made a significant contribution to the development of the disability. This is a principle which arises naturally from the plain meaning of the words "results from" and which is at least not more-embracing than the courts' concept of causation. It accords with the proposition that the breadth of the workers' protection for consequences of injuries was not intended to be reduced by the conversion from the common-law to the statutory system. It is, of course, the principle followed in previous decisions of this Tribunal...

[64] Tribunal jurisprudence has also addressed the issue of the threshold that must be established to conclude that the work-related compensable factors are actually “significant”. In this regard Tribunal *Decision No. 2062/01R*, stated at paragraph 18:

...In order to establish entitlement, it is not necessary to show that the workplace injury was the sole contributing factor, or even the predominant contributor. The workplace injury need only be a cause of the disability, providing that it makes more than a de minimus contribution. Even though the Tribunal uses the language of a “significant” contribution, Tribunal decisions illustrate that the test embodies the “material” contribution test developed in tort law. See for example, Decision Nos. 832/91 (October 7, 1992), Decision Nos. 228/02R (January 20, 2004) and 1645/99R (October 31, 2000). The use of the word “significant” is not intended to connote a higher standard for establishing causation in workplace injury cases.

[65] I adopt the Tribunal’s jurisprudence and line of reasoning, as stated in *Decision No. 915*, and *Decision No. 2062/01R*, as recited above. According to the jurisprudence, work-related factors are only deemed to make a “significant contribution” to the injury for which entitlement is being claimed, if the contribution of the work-related factors is more than the “de minimus” threshold. In other words, the contribution by work-related factors will be considered significant if the decision-makers conclude that the presence of such factors “made a difference” to the outcome of the medical condition for which entitlement is being claimed.

[66] Taking this approach in the circumstances of the instant appeal, I conclude that the worker’s work-duties did not make a significant contribution to her bilateral trigger fingers condition as related to Claim A for the following reasons.

[67] Claim A was initially established for bilateral CTS, which was the predominant clinical concern in the worker’s case in 2003 and 2004. Thereafter, trigger fingers were mentioned intermittently as affecting the worker over the years. The worker was assessed by Dr. Freiberg, a hand specialist, on September 16, 2004 where the worker complained of pain in her finger joints. Dr. Freiberg examined her, along with reviewing her medical history, and felt that the worker had “cervical problems such as neurogenic TOS and cervical radiculopathy” and “early degenerative changes in the IP and MCP joints of the thumb”. His report did not suggest that the worker had a work-related trigger finger condition. Dr. Handelsman later assessed the worker on November 8, 2004 and noted she had some triggering of her right thumb and received an injection “earlier in the week” but still gets “triggering involving her thumb”. He diagnosed “triggering involving her thumb” and provided another injection to the worker. Dr. Handelsman noted on March 3, 2005 that the injection “settled” the worker’s triggering of her right thumb. She then saw Dr. Martin, a plastic surgeon, on November 1, 2005 who noted the worker had “right trigger thumb” and “repeat triggering” so surgery was discussed on that occasion.

[68] Several years later, Dr. Vincent noted on February 17, 2011 that the worker presented with “trigger of her left fourth digit”, which he diagnosed as “stenosing tenosynovitis”. Dr. Weinberg diagnosed “bilateral carpal metacarpal arthritis in the thumbs” on May 17, 2011, which he also felt was a pre-existing condition “aggravated by the recent slip and fall”. However, I do not have jurisdiction in the present Tribunal appeal concerning whether the worker’s finger-related problems were aggravated by the fall under Claim B. Dr. McKee noted on July 5, 2011 that the worker had “some trigger fingers” and “CMC arthritis of her left thumb” with a history of right thumb release “seven to eight years ago”. Dr. McKee wrote a report dated December 10, 2012 noting the worker had “carpal tunnel and trigger fingers”, and that certain activities can aggravate/exacerbate the symptoms of trigger fingers. She noted that the worker’s circumstances also involved “a fall Jan. 18, 2011 (covered by WSIB) that aggravated her hands. Add to this her admitted shortness (increase likelihood of having wrists in greater extension on

the keyboard)...”. She also noted that “cumulative computer activities by people who put their job for many years ahead of their own well-being does creates patients... like [the worker].”

[69] I have also considered the explicit opinion of Dr. Somerville when he was asked by the Board about whether the worker’s trigger fingers surgeries of 2012 were related to the “original injury” under Claim A. His opinion dated May 16, 2012, was as follows:

Additionally, this case is for the injured worker's trigger finger surgery of April 2012. Occupational risk factors for trigger finger are much the same as for CTS, but especially involve prolonged and extensive gripping, such as using a power tool or musical instrument. Industrial workers, farmers and musicians are in high risk occupations. Individuals are also at greater risk if they have certain medical conditions, including rheumatoid arthritis, diabetes, and hypothyroidism. Trigger finger is more common in women. Most cases occur in individuals between 40 and 60 years old. Many with this common condition have few, or no, risk factors. [The worker] is not in a high risk occupation for trigger finger, nor did her work duties include exposure to known risk factors for this condition. The trigger finger surgery of April 2012 is also not related to the original injury of March 18 2003 and is therefore should not be considered a responsibility if this claim.

[70] On the whole, I find there to be little evidence suggesting a relationship of any sort between the worker’s intermittent problem with trigger fingers (of varying location and intensity over the years) and work-related factors within the context of Claim A. The only evidence that somewhat suggests the existence of such a relationship was Dr. McKee’s report in 2012, however, I do not find it to be persuasive. Dr. McKee there conflated the worker’s carpal tunnel syndrome and trigger fingers in her discussion, so much so that extracting an opinion about whether work-related factors specifically contributed to her trigger fingers was not possible. She also suggested that the events under Claim B materially affected the condition of the worker’s hands but I only have the jurisdiction to consider them under Claim A in the present Tribunal appeal. She also specifically discussed risk factors as it applied to factories, however, the worker’s work history with the employer in this case bore no resemblance to factory-related work. As to Dr. McKee’s comments about computer-use, there is no indication there that she had any understanding of the wide-range of tasks that the worker was responsible in her employment other than computer-use. While the worker used a computer for part of her day, she was not simply a data entry worker tied to her computer work-station for the majority of her day, as explained in the testimony of the employer’s witness at the Tribunal hearing. There was a variety of work, there were breaks, and there were changes in her job functions over the years. In this light, I give little weight to Dr. McKee’s report as it shed minimal light on whether the worker’s trigger fingers were related to Claim A.

[71] I also find it significant that the medical evidence that touched upon the condition of the worker’s fingers over the years also suggested the presence of varying degree of arthritis (including in 2004 and 2011), which is not compensable under the worker’s claim. There is also a suggestion of some involvement of the worker’s cervical and thoracic areas when she was assessed by Dr. Freiberg, the hand specialist, in 2004. While the worker was being seen by various doctors and specialists since 2003, medical reports did not explain to the Board, or to anyone else, that work-related factors were creating the worker’s trigger fingers condition. This suggests that there was a lack of nexus between the worker’s fluctuating condition and work-related factors. Similarly, the worker returned to regular duties following 2005 and did not require medical accommodation for trigger fingers following her work re-integration in late 2005. The worker’s work also varied over the material time, including moving from being a

support assistant, to trial scheduling and interpretation lead. The worker was then absent or accommodated for significant durations in later years due to non-compensable factors. Over this period of time, there was no evident correlation between the type of specific work duties that she was carrying out and the development of triggering-related symptomology or pathology. In my view, all of this suggests that the worker's trigger fingers were existing independent of any identifiable work-related factors.

[72] In this same light, I have also considered that Dr. Somerville noted that risk factors for a trigger condition are "prolonged and extensive gripping, such as using a power tool or musical instrument" but that the worker was "not in a high risk occupation for trigger finger, nor did her work duties include exposure to known risk factors for this condition". I have carefully considered the worker's work duties over the years, both in the worker's testimony and the employer's testimony at the Tribunal hearing, alongside the documentary record. I find that occupational risk factors for the development of a triggering condition were not present to any significant degree in the present case. The worker's work duties were not machine-paced. She did not handle any tools that required significant exertion of force or repetition. There was no extensive gripping involved. She handled materials that were light and clerical-related, as others carried boxes from the lower level when this was required. There were a variety of tasks that she accomplished over the years, including verbal tasks like interacting with other workers and speaking on the phone. There were also changes in the workplace over the years, such as the inclusion of Support Assistant B and electronic management systems implementation, which further lowered the amount of work that she had to manually perform with her hands. The fact that trigger fingers were bothering the worker to varying degree in that temporal context does not suggest any work-related genesis. I accept Dr. Somerville's medical opinion that the risk for trigger finger development was low in the worker's particular occupation. In my view, there is no contrary conclusion that can be supported on a balance of probabilities. I have finally considered that the only explicit medical opinion about compatibility of the worker's "trigger finger surgery of April 2012" to Claim A was from Dr. Somerville who noted it was "not related to the original injury of March 18 2003 and is therefore should not be considered a responsibility if this claim", which I also accept.

[73] On the whole, I find there to be a paucity of evidence to support a work-related connection between the development of the worker's bilateral trigger fingers claimed to be related to Claim A. There was no work-related or identifiable disablement resulting in trigger fingers in this case. The medical evidence did not suggest that the worker's condition was related to work-related factors in any meaningful way. Occupational risk factors for the development of her condition were largely absent, neither were they temporally correlated to the development of her condition in any meaningful way. As such, I find on a balance of probabilities that work-related factors were not a significant contributing role in the development of the worker's bilateral trigger fingers within the parameters of Claim A. As such, the worker's appeal is denied.

[74] I also reiterate that I do not have jurisdiction in the present Tribunal appeal to consider whether the worker's trigger fingers condition was aggravated under Claim B, something suggested in Dr. McKee's report in 2012.

DISPOSITION

[75] The worker's appeal is allowed, in part.

[76] The worker does not have ongoing entitlement for bilateral CTS, nor entitlement for right CTS surgery, under Claim A.

[77] The worker has claim entitlement under Claim B for an aggravation of a right-sided carpal tunnel syndrome, including entitlement for right CTS surgery dated November 4, 2011. The nature and duration of any additional benefits that the worker may be entitled under Claim B as a result of these determinations is remitted to the Board for future adjudication, subject to the usual rights of appeal.

[78] The worker does not have entitlement for bilateral trigger fingers as related to Claim A.

DATED: April 7, 2017

SIGNED: L. Petrykowski